SpeechLP®

Research Foundation & Logic Model

ESSA Tier 4 “Demonstrates a Rationale” Evidence Packet

ESSA TIER 4

Demonstrates a Rationale

THIRD-PARTY REVIEW PENDING

Prepared by:

Mari Gottlieb, M.A., CCC-SLP
Cofounder and Co-CEO

Alissa Fox, M.S., CCC-SLP
Cofounder and Co-CEO

Shail Silver
Cofounder and Executive Chair

NOVEMBER 2026

ESSA TIER 4

Demonstrates a Rationale

THIRD-PARTY REVIEW PENDING

DEMONSTRATES A RATIONALE

ESSA Tier 4 Evidence Statement for SpeechLP

SpeechLP

This packet documents how SpeechLP's design is grounded in published research on speech sound development and intervention, describes the program's features and intended outcomes in a logic model, and outlines a planned study of its effects. Together these components address the Every Student Succeeds Act (ESSA) criteria for Tier 4, "**Demonstrates a Rationale**."

TO COMPLETE — Independent review. The OgStar packet includes a certificate signed by an independent research organization (LXD Research). SpeechLP does not have one yet, so this page is a self-attestation. Once a third-party reviewer (e.g., LXD Research, Digital Promise, ISTE, or a university partner) has evaluated this packet, replace this box with their recognition statement, reviewer name/title, signature, and review date. Do not add a reviewer seal or signature until the review is actually issued.

Reviewer name, credentials Organization	**DATE**

Executive Summary

SpeechLP is a mobile app, with a web version of its practice games, built by practicing speech-language pathologists, that gives children frequent, structured, and measured practice of speech sounds at home and at school. Through its companion SLP tool, SpeechTherapist.app, therapists assign that practice and see how it is going.

What SpeechLP does

An age-based screener for ages 3–9, about 15 words per level, identifies a child's tricky sounds. In short video lessons, two certified SLPs model each sound and the child practices saying it, followed by speaking games that draw on more than 1,000 target words across all 24 English consonants. Automated speech recognition gives feedback on every word, and progress is reported to parents, teachers, and SLPs.

Who it serves

Preschool and elementary-age children who are working on speech sounds, whether or not they have a diagnosed speech sound disorder. Many families use it alongside therapy; others use it before they have an SLP or while waiting for services. It does not replace an SLP evaluation. Some children in the initial study were outside the 3–9 range, since families enter ages and some use the games with younger or older children.

Why it should work

Speech sound disorders are common, affecting an estimated 2.1%–23% of children aged 4–6. Children whose speech difficulties persist, or occur alongside language difficulties, are at greater risk for later reading and spelling problems. Treatment is effective, and research consistently points to practice amount, frequency, and feedback as drivers of progress. Yet therapy time is limited and home practice is rarely measured. SpeechLP is designed to increase practice and make it visible (see the Research Summary and Logic Model).

Initial evidence

A descriptive retrospective study of 101 returning children found that app-scored accuracy on the same sounds rose from 69.4% to 74.9% between two consecutive months (+5.5 percentage points, 95% CI 0.4 to 10.4). The scoring is not yet validated against SLP judgment, and there was no comparison group, so this is not evidence of effectiveness. It is a starting point for the studies that follow (see Initial Evidence).

ESSA status

This packet documents a research-based rationale and logic model and a completed initial study, with prospective evaluation planned, addressing the requirements for **ESSA Tier 4, "Demonstrates a Rationale."** Independent third-party review is pending.

Evidence goals

1. **Validate scoring** against several SLP raters, acoustic measures, and parent judgments.
2. **Study home practice dose** using de-identified existing data.
3. **Test effectiveness** in prospective studies with a comparison group, building toward Tier 3 or Tier 2 evidence.

~15	1,000+	2	+5.5 pp
screener words per age level	target words, all 24 consonants	CCC-SLPs modeling every sound	app-scored accuracy, month 1 to 2 (n = 101)

Copyright © 2026 SpeechLP
All rights reserved.

Understanding ESSA Evidence

Educators search for high-quality, evidence-based interventions to strengthen grant applications, to support comprehensive and targeted schools, and to implement new programming. Evidence requirements under the Every Student Succeeds Act (ESSA) are designed to help states, districts, and schools identify programs, practices, products, and policies that work across various populations.

Educational programs document their evidence of design, effectiveness, and impact in order to be eligible for federal funding. ESSA defines four levels of evidence: strong (Tier 1), moderate (Tier 2), and promising (Tier 3) evidence of effectiveness, or a program that demonstrates a rationale to be effective (Tier 4). The U.S. Department of Education's non-regulatory guidance describes Tier 4 as an activity with a well-specified logic model informed by research or evaluation, accompanied by ongoing efforts to examine its effects (U.S. Department of Education, 2016).

This packet addresses the requirements for Tier 4:

- ✓ Documentation of how the product's design relates to intended outcomes, with corresponding published academic research
- ✓ A logic model describing the product's resources, activities, outputs, and outcomes
- ✓ An initial descriptive study has been completed (Silver et al., 2026), and a prospective evaluation is planned (see "Planned Evaluation")
- ★ A third-party research organization reviews the documentation for ESSA validation [pending]

ESSA TIER 4

Demonstrates a Rationale

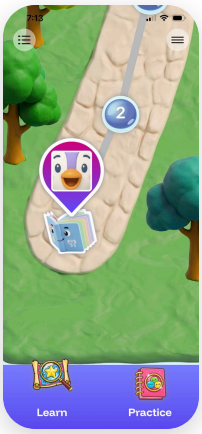
THIRD-PARTY REVIEW PENDING

When speech-language tools are built on what is known about how children acquire and practice speech sounds, families, teachers, and SLPs can extend high-quality practice well beyond the therapy room. This packet maps each SpeechLP feature to that research and lays out how its effects will be measured.

TO COMPLETE: Replace with a quote from the independent reviewer once the review is issued.

Copyright © 2026 SpeechLP
All rights reserved.

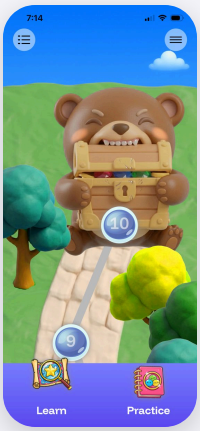
SpeechLP Supports Speech Sound Development

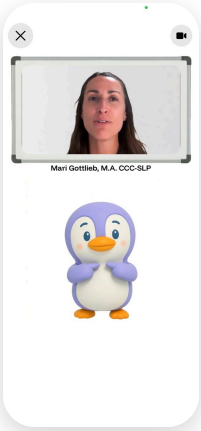


SpeechLP is a mobile app, with a web version of its practice games, that helps children practice clear speech sounds between, or in addition to, sessions with a speech-language pathologist. A brief age-based screener of about 15 words per age level identifies which sounds are tricky for a child. The app then builds a guided learning path of short SLP-led video lessons and playful practice games drawn from a library of more than 1,000 target words. SLPs use the companion **SpeechTherapist.app** to assign practice to students and see their progress.

Screen, Then Practice What Matters

Children begin with a short screener matched to their age, from 3 to 9, with about 15 words that sample the speech sounds expected by that age in initial, medial, and final word positions. Results determine which sounds the child practices, so time is spent on the sounds that need attention rather than on sounds already mastered.





Taught by Licensed SLPs, Practiced Through Play

Each step of the learning path opens with a short video lesson from a certified speech-language pathologist. The videos feature Mari Gottlieb, M.A., CCC-SLP, and Alissa Fox, M.S., CCC-SLP, who both model each target sound; Mari delivers the instructional cueing, which Alissa wrote. Children practice producing the sound along with the model, then practice their target sounds in words through games such as *Beary Hungry*, *Baby Talk*, *Chicken Little*, and *Pilot Penguin*.

Copyright © 2026 SpeechLP
All rights reserved.

SpeechLP

User Voices

Parents and speech-language pathologists are the people who support children's practice day to day, and the best judges of whether a practice tool fits their lives and caseloads. This section shares their experiences with SpeechLP.

TO COMPLETE — Real testimonials only. Replace each placeholder below with a genuine quote, collected with written permission to publish. Use first name and role (e.g., "Jessica, SLP, Ohio").

C

[Carrie's quote: paste her exact words here.]
Carrie
SpeechLP parent, mom of a young child

[Quote from an SLP using SpeechLP to support their caseload, e.g., assigning home practice and reviewing progress in SpeechTherapist.app.]
Name
Speech-Language Pathologist, State

S

Copyright © 2026 SpeechLP
All rights reserved.

SpeechLP® Foundational Research Summary

Introduction

Speech sound disorders are among the most common communication difficulties of childhood. In a large population-based sample, Shriberg, Tomblin, and McSweeny (1999) found that about 3.8% of 6-year-olds had a speech delay. ASHA reports estimated speech sound disorder prevalence of 2.1%–23% at ages 4–6 and persistent speech sound disorder in about 3.6% of children at age 8 (ASHA, n.d.). Children with speech sound disorders are at increased risk for later difficulties with reading and spelling, particularly when difficulties persist or co-occur with language impairment (Anthony et al., 2011), and residual speech errors in school-aged children have been linked to social, emotional, and academic impacts (Hitchcock et al., 2015).

Effective speech intervention depends heavily on practice. Children need many accurate productions of a target sound, distributed over time, to make lasting gains (Maas et al., 2008; Williams, 2012). Yet the amount of practice a child receives in typical school-based service—often short, small-group sessions once or twice a week—is limited (Sugden et al., 2018). Treatment itself is effective: a meta-analysis of controlled studies with children aged 3–18 found medium-to-high effects on phonological accuracy (Kunnari et al., 2024). The challenge is giving children enough of it.

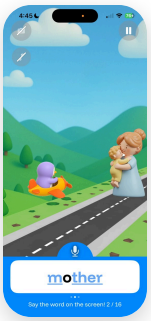
SpeechLP is a mobile app, with a web version of its practice games, that extends speech sound practice beyond the therapy session. It combines a brief screener, SLP-led video lessons, a large library of target words, and game-based word practice, with progress reports for parents, teachers, and SLPs. SpeechLP is designed to supplement, not replace, assessment and intervention by a licensed speech-language pathologist.

Research on SpeechLP's Key Differentiating Features

1. Age-Based Screening of Developmentally Expected Sounds

Children acquire English consonants in a broadly predictable developmental sequence (Smit et al., 1990). A review of 15 U.S. studies covering 18,907 children found that most English consonants were acquired by age 5;0 under a 90% acquisition criterion, with /ɹ/, /θ/, /ð/, and /ʒ/ among the later-acquired sounds (Crowe & McLeod, 2020). These population norms are not individual diagnostic deadlines, but they indicate which sounds a child of a given age would typically be expected to produce. Identifying which sounds are in error is the starting point for selecting intervention targets (McLeod & Baker, 2017).

SpeechLP has screeners for each age from 3 to 9, each about 15 words long, that sample the speech sounds children are expected to produce by that age. Ages 3, 4, and 5 each have their own set of sounds, reflecting the period when consonants are still being acquired; ages 6 through 9 share one set, since most English consonants are acquired by about age 5 (Crowe & McLeod, 2020). Each screener samples sounds in initial, medial, and final word positions. Target words were selected by SpeechLP's cofounding SLPs, Alissa Fox, M.S., CCC-SLP, and Mari Gottlieb, M.A., CCC-SLP, with the goal of using higher-frequency words that young children are likely to know and that are easy to depict as pictured objects, so that the screener measures speech production rather than vocabulary. Results personalize the learning path so each child practices the specific sounds that are difficult for them. Screener results are shared in a report that parents can bring to their child's SLP or teacher, supporting earlier conversations about whether a full evaluation is warranted. The screener is not a diagnostic assessment and does not replace an evaluation by a licensed SLP.



A screener card: the child says the pictured word aloud (word 2 of 16).

OPTIONAL: Add a screenshot of the screener results report, and list the sounds each age set includes.

2. Expert Modeling Through SLP-Led Video Lessons

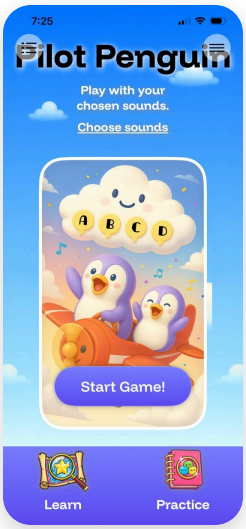
Learning a new speech movement benefits from clear instruction and a model to imitate. Motor learning research applied to speech emphasizes pre-practice—explaining and demonstrating how a sound is made—before a child begins repeated practice (Maas et al., 2008). Video modeling is also a well-established, effective instructional technique for children (Bellini & Akullian, 2007).

Each step of the SpeechLP learning path opens with a brief video in which a certified speech-language pathologist explains and demonstrates the target sound. The videos feature Mari Gottlieb, M.A., CCC-SLP, and Alissa Fox, M.S., CCC-SLP, who both provide models; Mari delivers the instructional cueing that guides children's productions, which Alissa wrote. During each lesson, the child practices producing the target sound on its own, imitating the SLP's model, before moving on to words. Because lessons are recorded, every child receives a consistent, expert model, and families without easy access to an SLP still see correct placement and production.

3. High-Dose Practice From a Library of 1,000+ Words

The number of practice trials matters. Children with speech sound disorders who produce more targets per session tend to make greater gains (Edeal & Gildersleeve-Neumann, 2011), and systematic reviews identify treatment intensity as a key ingredient of effective speech intervention (Kaipa & Peterson, 2016). How practice is distributed also matters: in a randomized study of preschool children receiving the multiple oppositions approach, treatment three times weekly produced greater phonological gains than treatment once weekly when cumulative intervention intensity was held constant (Allen, 2013), and in children with childhood apraxia of speech, twice-weekly motor speech treatment for 10 weeks outperformed once-weekly treatment on articulation and functional communication (Namasivayam et al., 2015). Motor learning research adds that practice amount, distribution, variability, and feedback schedule may affect retention and transfer (Maas et al., 2008).

SpeechLP draws from a library of more than 1,000 words, organized by sound and word position, so children can produce many different exemplars of their target sound rather than repeating the same small set. Practice in SpeechLP currently takes place at the sound level, in the video lessons, and at the word level, in the games. Treatment commonly progresses from establishing a sound to using it in more complex contexts and in conversation (ASHA, n.d.; McLeod & Baker, 2017), and Speech Motor Chaining applies motor learning principles through structured practice that builds from syllables to longer, self-generated utterances (Preston et al., 2019).



Children choose their target sounds, then practice them across many different words (Pilot Penguin).

4. Motivating, Purposeful Game-Based Practice

Repetitive drill can be hard for young children to sustain. A review of empirical studies found that gamification generally produces positive effects on engagement and motivation, with outcomes depending on the design and context (Hamari et al., 2014). Studies of tablet-based speech practice games for children have reported high engagement and production gains comparable to traditional paper-based practice (Hair et al., 2021), and computer-based approaches are a growing part of speech sound intervention (Furlong et al., 2017).

In SpeechLP, every game is built around speaking. In *Beary Hungry* the child feeds the bear by saying the word; in *Baby Talk* and *Chicken Little* the child must "say it right to make it pass." Children can choose which sounds to play with, progress along a visual map with numbered levels and rewards, and celebrate success with animated characters. These elements are designed to increase the number of productions a child is willing to attempt in a session.



Every game requires the child to produce their target sounds to advance.

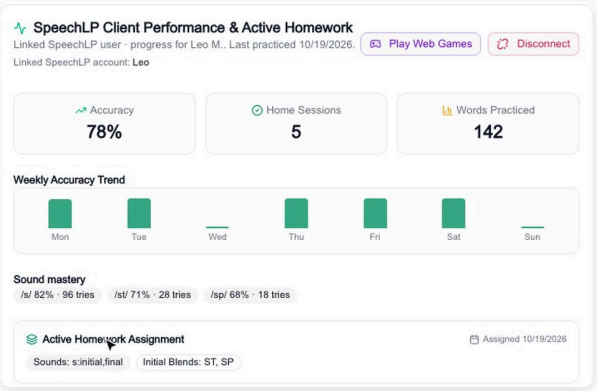
Automated feedback on every word. Each practice card shows a pictured target word and automatically plays a spoken model of it, which the child can replay as often as needed before speaking. Automated speech recognition then evaluates the child's production against the card's target sound, and a successful production advances the game (Silver et al., 2026). Automated feedback in structured speech practice is an active research area. In a single-case experiment, listener-rated /ɹ/ accuracy in practiced utterances was higher immediately after 30 minutes of AI-assisted Speech Motor Chaining practice than before it (Benway & Preston, 2024), and automated tools for analyzing children's speech are a growing field of study (McKechnie et al., 2018). In Swedish children aged 4–10, a speech recognition model rated single words within about 5 percentage points of an expert's own re-ratings on accuracy (Getman et al., 2022), and automated assessment has shown promise in Korean and Mandarin-speaking children (Kim et al., 2025; Lin et al., 2024); none has yet been validated for English home practice. In a single-case study of a self-supervised home speech app, two of four children showed slight improvement on trained words, without improvement on untrained words (Strömbergsson et al., 2026). SpeechLP's acceptance model has not yet been validated against SLP ratings; that validation is the first planned study, and SpeechLP is working to fine-tune the model, with research results guiding where it needs to improve. Audio is processed on the device and never stored; only text transcripts of what the app recognized are kept.

5. Connecting Home, School, and the SLP Through Assignment and Progress Data

Parents and caregivers can play a meaningful role in speech intervention. A review of parent involvement in intervention for speech sound disorders found that parents are frequently involved in delivering home practice and that such involvement can support outcomes (Sugden et al., 2016). In related areas of communication, parent-implemented interventions have shown positive effects on children's language skills (Roberts & Kaiser, 2011). ASHA's national outcomes data found that three times as many children with severe speech sound disorders made significant gains in intelligibility when they received 21–30 sessions, twice weekly, with an SLP-directed home program (ASHA, 2024). Yet home practice is rarely measured: surveys of SLPs have not quantified how often it is assigned (Sugden et al., 2018; Tambyraja, 2020), and intervention studies seldom report what it involved (Sugden et al., 2016).

SpeechLP produces reports designed to be read by parents, teachers, and SLPs. Reports summarize screener results and the child's practice and progress over time, giving the SLP information about home practice that would otherwise be unknown, and giving families clear, concrete next steps. Because the app is guided by SLP lessons, parents and teachers do not need clinical training to support practice.

For SLPs, SpeechLP connects to **SpeechTherapist.app**, a companion web app built for the SLP workflow. SLPs invite a caregiver, assign target sounds, word positions, and blends for home practice, and track home sessions, words practiced, and accuracy overall and by sound. Every scored practice item is logged with the word attempted, the sounds recognized, whether it was accepted, and a timestamp, so practice outside the therapy room is measured rather than assumed, and the SLP can adjust instruction based on data.



SpeechTherapist.app progress view (demo account): accuracy, home sessions, words practiced, weekly trend, sound-by-sound accuracy, and the active assignment.



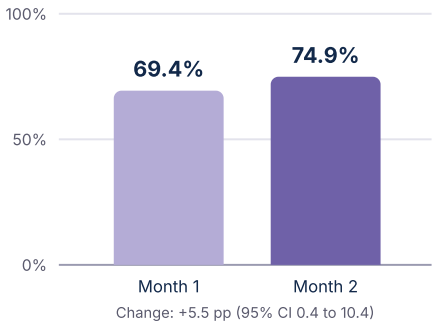
Children move through a guided learning path and celebrate their progress.

Initial Evidence: A Descriptive Retrospective Study

SpeechLP published a retrospective observational study of routine practice records from January 22 through October 7, 2026 (Silver et al., 2026). All authors are affiliated with SpeechLP. Shail Silver, Cofounder and Executive Chair, performed the initial analysis, with feedback from Jabali Ndungu, Head of Engineering, and Matthew Siu, Lead iOS Developer, who also reviewed the data analysis, statistical methods, and numerical results with him. Mari Gottlieb, M.A., CCC-SLP, and Alissa Fox, M.S., CCC-SLP, Cofounders and Co-CEOs, reviewed the clinical terminology, interpretation, and relevance to speech-language pathology practice. The study followed 101 returning child profiles (55 recorded as ages 2–5 and 33 as ages 6–8) across two consecutive 30-day periods and compared accuracy on identical sound targets within each profile.

What the study found

- Across 660 matched profile-and-sound pairs (11,902 practice cards), mean accepted-card accuracy rose from **69.4% to 74.9%**, a change of **+5.5 percentage points (95% CI 0.4 to 10.4)**.
- 54 of 101 profiles (53.5%) had higher second-month averages, 34 (33.7%) lower, and 13 (12.9%) unchanged; of 660 individual sound comparisons, 278 rose, 177 fell, and 205 were unchanged.
- Sixteen of 21 reportable sound means were higher in Month 2, including /k/ (+14.4 pp), /z/ (+16.0 pp), and /t/ (+11.1 pp); five were lower, including /ɹ/ (−3.3 pp).
- Practice declined in Month 2: mean sessions per profile fell from 6.73 to 3.44 and mean practice days from 4.70 to 2.71.



Mean accepted-card accuracy on matched sounds, 101 profiles (each profile weighted equally). Source: Silver et al. (2026).

TO CONFIRM (Shail, Matt, Jabali): Check every number on this page against the published study before sharing.

How to interpret these results

These findings are preliminary and descriptive, and the study's authors state their limits directly. Accuracy reflects the app's automated speech recognition decisions, which have not been validated against SLP ratings, and scoring changed over the study period without adjustment for app version. A card counted as accepted if any attempt succeeded, so scores can reflect repeated tries and listening to the model. The cohort was selected for returning use, there was no comparison group, and many children were in an age range of active sound acquisition, so the change cannot be attributed to SpeechLP practice or interpreted as clinical articulation improvement. The study did not test untrained words or connected speech. Under ESSA, this study does not meet the requirements for Tier 3 (promising) evidence; it is reported here as part of the rationale and as the reference point for the prospective evaluation that follows.

Full manuscript and aggregate data (CC BY 4.0): speechlp.com/research/articulation-practice-two-months · https://doi.org/10.5281/zenodo.23932485

Planned Evaluation

SpeechLP is committed to building evidence of its effectiveness beyond Tier 4 and has published an invitation to university research partners. Two starting studies are proposed. In each, SpeechLP provides practice data, the app platform, engineering support, and recruitment through connected SLPs; the partner leads study design, ethics approval, and analysis. The sequence is deliberate: first validate the app's scoring against SLPs, then use the validated platform to study practice and progress. Validation results will also guide fine-tuning of the app's model.

1. Scoring validation (starter pilot)	Compare the app's scores with expert SLP ratings, acoustic measures, and parents' correct/incorrect judgments during home practice, across children with a range of speech motor skill. This replicates in English the human-versus-machine design of Getman et al. (2022), with several SLP raters instead of one and the same metrics (accuracy, unweighted average recall, mean absolute error), and adds a parent comparison. Using several raters matters because expert listening has limits of its own (Meyer & Munson, 2021; Allison et al., 2025), and combining expert listening with instrumental measures is recommended (Namasivayam et al., 2025).
2. Home practice dose (existing data)	Test whether the amount, spacing, and accuracy of home practice predict gains, and how closely families complete SLP-assigned practice, using de-identified data under a data use agreement.
Longer-term studies	Once scoring is validated: randomized comparisons of feedback type and frequency within the app, and prospective outcome studies with a comparison group and independent, functional measures such as percentage of consonants correct (Shriberg et al., 1997) and the Intelligibility in Context Scale (McLeod et al., 2012), plus stimulability and self-monitoring.
Research partner	SpeechLP is actively seeking a university research partner; finding a good fit is the first step. The studies above are proposals to be finalized with that partner.

Why generalization must be measured directly

Gains on practiced items do not guarantee carryover. In a three-child home-delivered articulation study, transfer across tested settings exceeded transfer to untrained words (Costello & Bosler, 1976). Self-monitoring has been studied as a way to support generalization, although its relationship with conversational gains is variable (Koegel et al., 1986; Shriberg & Kwiatkowski, 1990). A meta-analysis of 12 treatment studies found that treating more complex sounds also improved untreated, simpler sounds (Maggu et al., 2021), but this does not establish transfer from every form of app practice. Outcome studies will therefore measure untrained words and connected speech rather than relying on in-app scores.

Research Partnership Terms

- **Independence.** Partner researchers control the analysis and can publish regardless of results. SpeechLP reviews manuscripts within 30 days only to protect participant privacy and correct factual errors about the product, not to approve findings.
- **Agreements.** A data use agreement covers access, privacy, storage, and ownership of any models or methods a partner develops using SpeechLP data. Ethics approval runs through the partner's institutional review board.
- **Privacy.** Data is shared de-identified, with small groups suppressed. Account and child identifiers never leave SpeechLP, and audio is never stored.
- **Support.** App licenses for study participants, engineering time for data exports, and co-authorship for SpeechLP clinicians where contributions warrant it. Funding can be discussed once a specific study is scoped.
- **Transparency.** SpeechLP's cofounders have a financial interest in the app, and this is disclosed in all outputs.

About the Team

SpeechLP was founded and is led by practicing speech-language pathologists.

Alissa Fox, M.S., CCC-SLP — Cofounder and Co-CEO. SLP since 2004, treating children's speech sound disorders, motor speech delays, and language disorders in schools and via teletherapy, with a focus on assistive technology. Vice President for Schools at the Iowa Speech-Language-Hearing Association; licensed in Pennsylvania and Iowa. Wrote the cueing used in SpeechLP and provides models in its video lessons.

Mari Gottlieb, M.A., CCC-SLP — Cofounder and Co-CEO. Private practice owner and SLP since 2017, treating children's speech sound and language disorders in schools and via teletherapy, and cognitive-communication impairments during nearly six years at a brain injury and concussion clinic. SPEAK OUT!-trained; brings a prior advertising career to family engagement and app design; licensed in Colorado and Washington. Delivers the instructional cueing and provides models in SpeechLP's video lessons.

Shail Silver — Cofounder and Executive Chair. **Jabali Ndungu** — Head of Engineering. **Matthew Siu** — Lead iOS Developer. Both provide data and analysis support.

> **Next step.** Districts, schools, and university researchers interested in partnering on SpeechLP research are invited to a 30-minute call. Contact Alissa Fox, M.S., CCC-SLP, at **alissa@speechlp.com**.

Implementation and Instructional Support

SpeechLP runs on iPhone and iPad, and its practice games are also available in a web browser, so children can practice at home, in the classroom, or in the therapy room. Families can begin with the screener in minutes, and the guided learning path tells the child what to do next, minimizing setup for adults.

SpeechTherapist.app

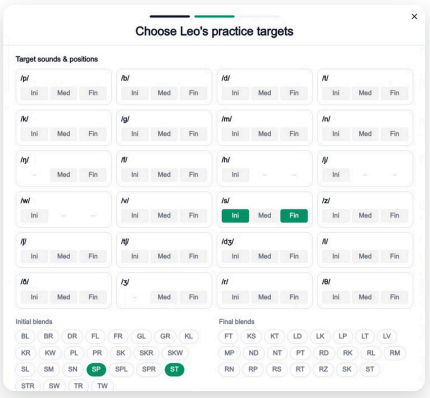
The SLP workflow. SpeechTherapist.app gives speech-language pathologists one place to manage SpeechLP practice for their caseload:

- **Invite and assign:** invite a caregiver with a connection code, then assign target sounds, word positions, and blends.
- **Monitor:** track games played, words practiced, and accuracy overall and by sound.
- **Plan:** use that data to decide what to target next and spend session time on new skills.

Privacy. SpeechLP complies with COPPA, FERPA, HIPAA, and PIPEDA. Audio is processed on the device and never stored; only text transcripts of what the app recognized are saved to SpeechLP's servers.

TO CONFIRM (Shail, Jabali): Verify that the COPPA, FERPA, HIPAA, and PIPEDA compliance statements and the on-device audio processing claims are accurate before external sharing. These statements also appear in the school implementation model and the partnership terms.

A recommended model for school implementation follows on the next page.



Assigning home practice in SpeechTherapist.app (demo account): the SLP picks target sounds, word positions, and consonant blends, and the assignment is sent to the family's SpeechLP app.

When educators use data from digital practice to guide instruction, they can better target what each child needs next. Together, SpeechLP and SpeechTherapist.app are designed to give SLPs visibility into home practice so that session time can focus on new skills, and so that families and teachers can reinforce the same targets.

Recommended School Implementation Model

SpeechLP is not yet used in schools. Below is our recommended school model, based on its home-practice workflow and this packet's research; details will be confirmed with each district.

Who uses it	The school SLP assigns practice through SpeechTherapist.app. Students practice at school (for example, at the start of a speech session or during a short independent block) and at home with a caregiver.
Recommended dosage	Short sessions of about 5–10 minutes, 3–5 days a week, in addition to direct services. Frequent, distributed practice is supported by research on treatment frequency (Allen, 2013; ASHA, 2024).
Onboarding and training	A live virtual onboarding of about 45 minutes for SLPs (setup, assigning practice, reading progress data), short on-demand videos, and a one-page caregiver guide in English and Spanish.
Support	Email support (target response within one business day) and first-semester check-ins.
Rostering and sign-on	Initially, SLPs add students to their caseload in SpeechTherapist.app, individually or by spreadsheet upload. For district rollouts, single sign-on and automated rostering (for example, through Clever or ClassLink) are recommended.
Devices	iPhone or iPad, or a computer for the web games, each with a working microphone; school iPads and laptops are suitable. A headset microphone and a quiet space improve speech recognition.
Data privacy	Covered on the previous page. Districts can expect a signed student data privacy agreement, such as the Student Data Privacy Consortium's National Data Privacy Agreement or a state-specific agreement.

TO CONFIRM before submission: Recommendations, not commitments; keep only what SpeechLP will offer. **Alissa:** review the full model for feasibility. **Matt:** devices and web-version features. **Shail:** privacy agreements.

Conclusion

SpeechLP brings together several research-supported ingredients of speech sound intervention—age-based selection of target sounds, expert modeling, high-dose practice across many words, motivating game-based practice, and family involvement—in a tool children can use between SLP sessions. An initial descriptive study found higher automated accuracy scores on matched sounds in a second month of use among returning users, and the logic model and planned evaluation provide a clear rationale for its expected impact and a path to building stronger evidence of effectiveness.

SpeechLP Logic Model

PROBLEM STATEMENT

Speech sound disorders are among the most common communication difficulties of childhood, and untreated or persistent errors can affect a child's intelligibility, literacy, confidence, and participation in school. Progress depends on frequent, accurate practice, but children typically see an SLP for limited time each week, SLP caseloads are large, and many families lack access to services or do not know how to practice at home. Practice materials are often repetitive and fail to hold children's attention. SpeechLP helps close this gap with an engaging, SLP-guided practice app that identifies each child's tricky sounds and delivers personalized, high-dose practice at home or school, with progress reports that keep parents, teachers, and SLPs connected.

RESOURCES

- Age-based screeners for ages 3–9, about 15 words each, sampling initial, medial, and final positions
- Short video lessons with models by Mari Gottlieb, M.A., CCC-SLP, and Alissa Fox, M.S., CCC-SLP; cueing written by Alissa and delivered by Mari
- Library of 1,000+ target words organized by sound and position
- Word-level speech practice games (e.g., Beary Hungry, Baby Talk, Chicken Little, Pilot Penguin)
- Guided learning path with levels and rewards
- Progress reports for parents, teachers, and SLPs
- SpeechTherapist.app, the SLP companion for assigning practice and monitoring progress
- iPhone/iPad app and web version of the games, used with a microphone

STRATEGIES AND ACTIVITIES

- Child completes the screener; results personalize the learning path to the child's target sounds.
- Child watches an SLP model how to produce the target sound and practices saying it before practicing words.
- Each practice card plays a spoken model of the word, which the child can replay before producing it.
- Child produces many different target words in short, game-based sessions.
- Child chooses sounds to play with and advances along a map, earning rewards.
- Parents, teachers, and SLPs review reports and reinforce the same targets.
- SLPs assign target sounds and practice in SpeechTherapist.app, then review each student's progress to plan sessions.

Copyright © 2026 SpeechLP
All rights reserved.

OUTPUTS

- Each child's tricky sounds are identified relative to age expectations.
- Children view expert SLP models of each target sound.
- Children complete a high number of target productions per session across many different words.
- Children complete practice sessions regularly at home and/or school.
- Parents, teachers, and SLPs receive regular progress reports.
- SLPs have current data on each student's assigned practice and progress across their caseload.

SHORT-TERM AND INTERMEDIATE OUTCOMES

Improved accuracy of target sounds in practiced words. Increased awareness of how target sounds are produced. Greater willingness to practice and increased practice frequency. Earlier identification and referral of children who may need an SLP evaluation.

As children progress through SpeechLP, they are expected to show improved accuracy of target sounds in connected speech and conversation, generalization to untrained words, improved overall intelligibility, increased confidence when speaking, stronger home–school–SLP collaboration, and more efficient use of SLP session time.

LONG-TERM OUTCOMES AND IMPACTS

- Age-appropriate speech sound production and intelligibility
- Fewer residual speech errors persisting into later grades
- Shorter time to dismissal from speech services
- Reduced risk of related literacy difficulties
- Greater confidence and participation in classroom and social communication
- Increased family capacity to support communication development

ASSUMPTIONS

- An adult (parent, teacher, or SLP) is available to support the child as needed.
- Access to an iPhone, iPad, or computer with a working microphone and a reasonably quiet practice space.
- Access to the internet.
- Children with suspected speech sound disorders are also evaluated and served by a licensed SLP where available.

Copyright © 2026 SpeechLP
All rights reserved.

References

Allen, M. M. (2013). Intervention efficacy and intensity for children with speech sound disorder. *Journal of Speech, Language, and Hearing Research, 56*(3), 865–877. https://doi.org/10.1044/1092-4388(2012/11-0076)

Allison, K. M., Millington, M., & Grimm, A. (2025). Can we trust our ears? How accurate and reliable are speech-language pathologists' estimates of children's speech intelligibility? *American Journal of Speech-Language Pathology, 34*(2), 853–867. https://doi.org/10.1044/2024_AJSLP-24-00247

American Speech-Language-Hearing Association. (n.d.). *Speech sound disorders: Articulation and phonology* [Practice Portal]. https://www.asha.org/practice-portal/clinical-topics/articulation-and-phonology/

American Speech-Language-Hearing Association. (2024). *Dosage and home practice matters: Improving intelligibility in children with severe speech sound disorders* [National Outcomes Measurement System data snapshot]. https://www.asha.org/siteassets/noms/noms-speech-sound-disorders-data-snapshot.pdf

Anthony, J. L., Aghara, R. G., Dunkelberger, M. J., Anthony, T. I., Williams, J. M., & Zhang, Z. (2011). What factors place children with speech sound disorders at risk for reading problems? *American Journal of Speech-Language Pathology, 20*(2), 146–160.

Bellini, S., & Akullian, J. (2007). A meta-analysis of video modeling and video self-modeling interventions for children and adolescents with autism spectrum disorders. *Exceptional Children, 73*(3), 264–287.

Benway, N. R., & Preston, J. L. (2024). Artificial intelligence–assisted speech therapy for /ɹ/: A single-case experimental study. *American Journal of Speech-Language Pathology, 33*(5), 2461–2486. https://doi.org/10.1044/2024_AJSLP-23-00448

Costello, J., & Bosler, S. (1976). Generalization and articulation instruction. *Journal of Speech and Hearing Disorders, 41*(3), 359–373. https://doi.org/10.1044/jshd.4103.359

Crowe, K., & McLeod, S. (2020). Children's English consonant acquisition in the United States: A review. *American Journal of Speech-Language Pathology, 29*(4), 2155–2169.

Edeal, D. M., & Gildersleeve-Neumann, C. E. (2011). The importance of production frequency in therapy for childhood apraxia of speech. *American Journal of Speech-Language Pathology, 20*(2), 95–110.

Furlong, L., Erickson, S., & Morris, M. E. (2017). Computer-based speech therapy for childhood speech sound disorders. *Journal of Communication Disorders, 68*, 50–69.

Getman, Y., Al-Ghezi, R., Voskoboinik, E., Grósz, T., Kurimo, M., Salvi, G., Svendsen, T., & Strömbergsson, S. (2022). wav2vec2-based speech rating system for children with speech sound disorder. In *Proceedings of Interspeech 2022* (pp. 3618–3622). https://doi.org/10.21437/Interspeech.2022-10103

Hair, A., Ballard, K. J., Markoulli, C., Monroe, P., McKechnie, J., Ahmed, B., & Gutierrez-Osuna, R. (2021). A longitudinal evaluation of tablet-based child speech therapy with Apraxia World. *ACM Transactions on Accessible Computing, 14*(1), 1–26.

Hamari, J., Koivisto, J., & Sarsa, H. (2014). Does gamification work? A literature review of empirical studies on gamification. *Proceedings of the 47th Hawaii International Conference on System Sciences*, 3025–3034.

References (continued)

Hitchcock, E. R., Harel, D., & Byun, T. M. (2015). Social, emotional, and academic impact of residual speech errors in school-aged children: A survey study. *Seminars in Speech and Language, 36*(4), 283–294.

Kaipa, R., & Peterson, A. M. (2016). A systematic review of treatment intensity in speech disorders. *International Journal of Speech-Language Pathology, 18*(6), 507–520.

Kim, D. H., Jeong, J. W., Kang, D., Ahn, T., Hong, Y., Im, Y., Kim, J., Kim, M. J., & Jang, D.-H. (2025). Usefulness of automatic speech recognition assessment of children with speech sound disorders: Validation study. *Journal of Medical Internet Research, 27*, e60520. https://doi.org/10.2196/60520

Koegel, L. K., Koegel, R. L., & Costello Ingham, J. (1986). Programming rapid generalization of correct articulation through self-monitoring procedures. *Journal of Speech and Hearing Disorders, 51*(1), 24–32. https://doi.org/10.1044/jshd.5101.24

Kunnari, S., Sanduvete-Chaves, S., Chacon-Moscoso, S., Alves, D. C., Ozbič, M., Petinou, K., Tolonen, A.-K., Zajdó, K., Frizelle, P., Murphy, C.-A., Saldana, D., & Laasonen, M. (2024). Intervention studies with group design targeting expressive phonology for children with developmental speech and language disorder: A systematic review and meta-analysis. *International Journal of Language & Communication Disorders, 59*(6), 2686–2705. https://doi.org/10.1111/1460-6984.13110

Lin, Y.-S., Tseng, S.-C., & Jang, J.-S. R. (2024). Leveraging phonemic transcription and Whisper toward clinically significant indices for automatic child speech assessment. In *Proceedings of Interspeech 2024*. https://doi.org/10.21437/Interspeech.2024-1887

Maas, E., Robin, D. A., Austermann Hula, S. N., Freedman, S. E., Wulf, G., Ballard, K. J., & Schmidt, R. A. (2008). Principles of motor learning in treatment of motor speech disorders. *American Journal of Speech-Language Pathology, 17*(3), 277–298.

Maggu, A. R., Kager, R., To, C. K. S., Kwan, J. S. K., & Wong, P. C. M. (2021). Effect of complexity on speech sound development: Evidence from meta-analysis review of treatment-based studies. *Frontiers in Psychology, 12*, 651900. https://doi.org/10.3389/fpsyg.2021.651900

McKechnie, J., Ahmed, B., Gutierrez-Osuna, R., Monroe, P., McCabe, P., & Ballard, K. J. (2018). Automated speech analysis tools for children's speech production: A systematic literature review. *International Journal of Speech-Language Pathology, 20*(6), 583–598.

McLeod, S., & Baker, E. (2017). *Children's speech: An evidence-based approach to assessment and intervention*. Pearson.

McLeod, S., Harrison, L. J., & McCormack, J. (2012). The Intelligibility in Context Scale: Validity and reliability of a subjective rating measure. *Journal of Speech, Language, and Hearing Research, 55*(2), 648–656. https://doi.org/10.1044/1092-4388(2011/10-0130)

Meyer, M. K., & Munson, B. (2021). Clinical experience and categorical perception of children's speech. *International Journal of Language & Communication Disorders, 56*(2), 374–388. https://doi.org/10.1111/1460-6984.12610

Namasivayam, A. K., Kent, R., Preston, J. L., Maassen, B. A. M., Hagedorn, C., Nip, I. S. B., McAllister, A., Wang, J., Hustad, K. C., Ménard, L., Bahar, N., Moore, J. G., Petrosov, J., & van Lieshout, P. (2025). Reevaluating the classification of pediatric speech sound disorders: A ground truthing perspective. *Frontiers in Human Neuroscience, 19*, 1700505. https://doi.org/10.3389/fnhum.2025.1700505

Namasivayam, A. K., Pukonen, M., Goshulak, D., Hard, J., Rudzicz, F., Rietveld, T., Maassen, B., Kroll, R., & van Lieshout, P. (2015). Treatment intensity and childhood apraxia of speech. *International Journal of Language & Communication Disorders, 50*(4), 529–546. https://doi.org/10.1111/1460-6984.12154

References (continued)

Preston, J. L., Leece, M. C., & Storto, J. (2019). Tutorial: Speech motor chaining treatment for school-age children with speech sound disorders. *Language, Speech, and Hearing Services in Schools, 50*(3), 343–355. https://doi.org/10.1044/2018_LSHSS-18-0081

Roberts, M. Y., & Kaiser, A. P. (2011). The effectiveness of parent-implemented language interventions: A meta-analysis. *American Journal of Speech-Language Pathology, 20*(3), 180–199.

Shriberg, L. D., & Kwiatkowski, J. (1990). Self-monitoring and generalization in preschool speech-delayed children. *Language, Speech, and Hearing Services in Schools, 21*(3), 157–170. https://doi.org/10.1044/0161-1461.2103.157

Shriberg, L. D., Austin, D., Lewis, B. A., McSweeny, J. L., & Wilson, D. L. (1997). The percentage of consonants correct (PCC) metric: Extensions and reliability data. *Journal of Speech, Language, and Hearing Research, 40*(4), 708–722. https://doi.org/10.1044/jslhr.4004.708

Shriberg, L. D., Tomblin, J. B., & McSweeny, J. L. (1999). Prevalence of speech delay in 6-year-old children and comorbidity with language impairment. *Journal of Speech, Language, and Hearing Research, 42*(6), 1461–1481.

Silver, S., Fox, A., Gottlieb, M., Siu, M., & Ndungu, J. (2026). *Children's articulation practice in SpeechLP across two months* [Preprint; retrospective observational study]. SpeechLP. https://doi.org/10.5281/zenodo.23932485

Smit, A. B., Hand, L., Freilinger, J. J., Bernthal, J. E., & Bird, A. (1990). The Iowa articulation norms project and its Nebraska replication. *Journal of Speech and Hearing Disorders, 55*(4), 779–798.

Strömbergsson, S., Edlund, E., Pettersson, M., Phan, N., & Kurimo, M. (2026). Self-supervised app-based speech training for children with speech sound disorder: A single-case experimental design study. *International Journal of Language & Communication Disorders, 61*(1), e70163. https://doi.org/10.1111/1460-6984.70163

Sugden, E., Baker, E., Munro, N., & Williams, A. L. (2016). Involvement of parents in intervention for childhood speech sound disorders: A review of the evidence. *International Journal of Language & Communication Disorders, 51*(6), 597–625. https://doi.org/10.1111/1460-6984.12247

Sugden, E., Baker, E., Munro, N., Williams, A. L., & Trivette, C. M. (2018). Service delivery and intervention intensity for phonology-based speech sound disorders. *International Journal of Language & Communication Disorders, 53*(4), 718–734. https://doi.org/10.1111/1460-6984.12399

Tambyraja, S. R. (2020). Facilitating parental involvement in speech therapy for children with speech sound disorders: A survey of speech-language pathologists' practices, perspectives, and strategies. *American Journal of Speech-Language Pathology, 29*(4), 1987–1996. https://doi.org/10.1044/2020_AJSLP-19-00071

U.S. Department of Education. (2016). *Non-regulatory guidance: Using evidence to strengthen education investments*. Washington, DC.

Williams, A. L. (2012). Intensity in phonological intervention: Is there a prescribed amount? *International Journal of Speech-Language Pathology, 14*(5), 456–461.

This packet was drafted with AI assistance and reviewed by the authors.



SpeechLP

SpeechLP helps children practice clear speech with SLP-led video lessons, a 1,000+ word library, and games they'll actually want to play.

For additional information about SpeechLP visit:
www.speechlp.com

SpeechTherapist.app

SLPs: assign practice and track student progress at **SpeechTherapist.app**

Research partnership inquiries: alissa@speechlp.com